

إسم المريض رابعياً : شاديه حسنى عبد العظيم

المحافظة : الجيزة

الحى / المدينة : يولاق الذكرور

العنوان : ؤ

تاريخ تسجيل الإستمارة : 21-09-2026

رقم البطاقة : 25911082400041 رقم التلفون : 01150295138

العمر(تاريخ الميلاد) : 08-11-1959 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

**قسم العلاج :
الاورام**

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات الاروماتاز [14161]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع:

التوقيع:

التوقع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / ۲ ----- / ۲ ----- / ۲. | ----- / ۲. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: شاديه حسني عبد العظيم National ID: 25911082400041	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



شادية
حماني عبد العظيم
ع ٩ الحي السادس - مجاورة السادس
٦ أكتوبر ثان - الجيزة

٢٥٩١١٠٨٢٤٠٠٠٤١

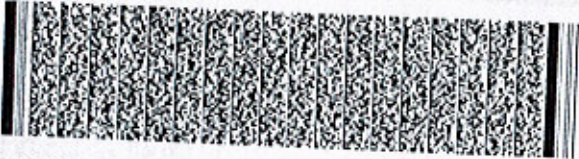
KD2457881

٢٠٢٣/٠٨ ٢٥٩١١٠٨٢٤٠٠٠٤١

ربة منزل

أنثى
متزوجة
صباحي يعقوب مجاهد مصطفى

البطاقة سارية حتى ٢٠٣٠/٠٨/١٨





جمهورية مصر العربية
بطاقة تحقيق الشخصية

شادية

حسنى عبدالعظيم

ع ٩ الحى السادس - مجاورة السادس

٦ أكتوبر - الجيزة

١٩٥٩/١

٢٥٩١١٠٨٢٤٠٠٠٤١

KD2457881



٢٠٢٣/٠٨ ٢٥٩١١٠٨٢٤٠٠٠٤١

ربة منزل

أنثى مسلمة متزوجة

صباحى يعقوب مجاهد مصطفى

البطاقة سارية حتى ٢٠٢٠/٠٨/١٨





Ministry of health
Central health laboratories
Pathology department

Report Number: m2324

Immunohistochemistry report

Female

الشيخ زايد

11/09/2023 11:42

13/09/2023 09:49

تاريخ استلام العينة

تاريخ التقرير

شائية حشوي مودال عظيم

65 Y

بعض

فخذ العينة

لتحليل

مسند في الشيخ زايد التخصصي

مسند في الشيخ زايد التخصصي

Primary Sample

Paraffin Block

Test

ER & PR & HER2 neu & Ki-67 (ER & PR: Harvey scoring system - Her2neu : CAP scoring system)

Comment

Technique

sample received was treated by - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody. - CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.
Sample received was treated by - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / XT.
Sample received was treated by - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result

ER: No nuclear staining of tumour cells (core biopsy)
PR: weak nuclear staining of 2 % of tumour cells (core biopsy)
HER2/neu protein over expression: strong continuous membrane staining of > 10 % of tumor cells. (core biopsy)
Ki67: positive nuclear staining of > 20 % of tumour cells (core biopsy)

Referral Lab

Comment

Conclusion

ER: NEGATIVE
PR: POSITIVE (SCORE 3/8)
HER2/NEU PROTEIN OVER EXPRESSION: POSITIVE (SCORE 3+)
KI-67: THE PROLIFERATION INDEX OF THE TUMOUR IS > 20%

Pathologist

دكتور/أ. ماهر

Lab Director

دكتور/أ. ماهر

9 El-shokh Rehan Street, Abdeldeen, Cairo
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Website: <http://www.healthlabs.net>
Email: pathologych@yahoo.com

مركز البحوث - القاهرة - مصر

This Test is Not Accredited By EGAC Nor Recognized By ILA

يتم الاعتماد وتحليل العينة لمدة 30 يوم من تاريخ استلامها

Immunohistochemistry

Page 1 of 1 [DOCTYPE: IHC]

Patient Name : الأستاذة / شادية حسني عبدالعظيم
Patient No. : 25911082400041
Path No. : 3963/24
Sex : Female
Age : 64 Y
Referring Doc :

Request No. : 13009159
Request Date : 16/10/2024
Verification Date : 03/11/2024
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer (post NACT) .
Nature of the specimen: Right MRM

Gross Examination:

- Procedure: Right MRM
- measured 22x20x4 cm with covering skin ellipse measured 20x14cm & attached axilla 9x6 cm .
- Tumor extent : An ill defined greyish white tumour bed site
- Size: 1.7x1x1 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 3 cm away from deep margin , 2 cm away from the skin .
- Lymph nodes number and size: Fibrofatty tissue piece measured 9x7x1.5 cm. Careful and repeated Axillary dissection entangling 7 L.Ns up to 1.7x1.5 cm.

Microscopic examination:

- Histological type: No residual viable tumor tissue detected
- Ductal carcinoma in situ: No.
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/7).
- Resection Margins: Free deep margin.
- Histologic response to therapy: Pathologic Complete response
- Additional pathological findings: Extensive necrosis and hyalinosis.

Diagnosis:

- Right MRM (post NACT) :
No residual viable tumor tissue detected.
- Free margins
 - Free nipple and areola

Verified by :
DR. Menna Nabil

Printing Date : 11/11/2024 13:47:57

Page 1 of 2

Printing By : marwa mohamed youss

Patient Name : الاسلام / شادية حسنى عبدالعظيم	Request No. : 13009159
Patient No. : 25911082400041	Request Date : 16/10/2024
Path No. : 3963/24	Verification Date : 03/11/2024
Sex : Female	Financial Category : أورام داء السل - فئة الدولة
Age : 64 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

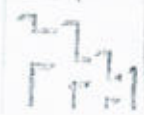
Pathology Report

- Free L.Ns (0/7).
- Pathologic Complete Response
- Stage : ypT0 , ypN0.

Dr. Salma Tarek,
Dr. Menna Nabil,

Verified by :
DR Menna Nabil

Printing Date : 11/11/2024 13:47:57	Page 2 of 2	Printing By : marwa mohamed youss
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Name	شادية حسني عبدالعظيم	Age	65 Y	Code	566-23
Referred by	أ.د/ حسين زكي	Date of receiving specimen	6/8/2023		
Patient ID	25911082400041	Sex	Female		
Clinical data	Right breast mass.				
Specimen	US guided biopsy.				

PATHOLOGY REPORT

GROSS: Core biopsy specimen, totally embedded.

Microscopic Picture: Sections examined reveal tissue cores infiltrated by malignant cells arranged singly and in clusters, with moderate tendency to form tubules. The neoplastic cells show moderate degrees of anaplasia. No in-situ component seen in this biopsy.

Diagnosis: *Right breast mass; US guided biopsy:*
" *Invasive duct carcinoma, Grade II.*

Consultant of Pathology Unit
Ass Prof. Dr.: Mohamed Sherif, MD
Dr.: Mostafa Fawzy, MD.



EXAMINER	Shadia Hosni abd alathem Hassan	DATE	Female
PATIENT ID	Q39720	TIME	1/8/2024 12:39:33 PM

Whole body ^{18}F -FDG PET/CT

Clinical History

- A patient presenting with right breast invasive duct carcinoma received neoadjuvant chemotherapy, referred for assessment.

Procedure:

- Whole body ^{18}F -FDG images from skull base to mid thighs. CT images of the same level.

Injected dose: 214.5 MBq	Injection time: 02:26	Acquisition time: 04:05
BGL: 106 mg/dl	Weight: 73 Kg	Liver SUVmean: 1.9
IV contrast: No	Oral contrast: water	Time/bed position
First PET/CT study.		

Opinion

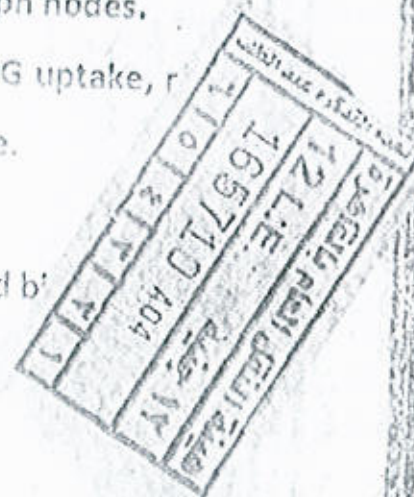
- Low grade metabolically active right breast neoplastic lesion associated with low grade metabolically active right axillary lymph nodes; as described.
- Multiple pulmonary nodules with no significant metabolic activity, metastatic nature is possible for close follow-up.



NAME	Shadia Hosni abd alathem Hassan	SEX	Female
MRN	Q39720	DATE	1/8/2024 12:39:33 PM

Findings:

- Right breast:
 - Low grade FDG uptake by lower outer quadrant spiculated mass with SUVmax 1.8 and measuring 2 X 1.4 cm in maximal axial diameter.
 - Low grade FDG uptake by overlying skin thickness with SUVmax 1.9 and measuring 17 mm in maximum thickness.
 - No chest wall invasion.
- Left breast: no FDG-avid masses.
- Lymph nodes:
 - Low grade FDG uptake by left axillary level I subcentimetric lymph node, SUVmax 1.5.
 - Low grade FDG uptake within small mediastinal and right external iliac lymph nodes.
 - Otherwise, no FDG avid pathologically enlarged lymph nodes.
- Lungs: bilateral pulmonary nodules with no significant FDG uptake, 7 mm nodule at the superior segment of left lower lung lobe.
- Bone marrow, no FDG-avid osseous lesions.
- Normal sized liver with no FDG-avid focal lesions. No dilated b



Kast El-Almy Center of Radiation Oncology and
Nuclear Medicine

Faculty of Medicine, Cairo University

"HEMROCK"

Nuclear Medicine Unit



مركز كاست الالمى

لطب الأورام والحساسية الذرية

شعبة الطب - جامعة القاهرة

(مدمرك)

Report

Patient Name	Shadia Hosni abd alathem Hassan	Gender	Female
Patient ID	Q39720	Date	1/8/2024 12:39:33 PM

- The spleen, kidneys, pancreas, adrenals, pelvic organs, aorta and IVC appear unremarkable.

Dr. Osama El-Mogy

Dr. Mona Saied, M.D



Patient Name: Shadia Hosny Abd Al Azeem	Date : 21/10/2024
Exam : C.T ABDOMEN & PELVIS	Age : 65 Year(s)
Medical ID: 3890/2023	Pat. ID: 53786

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right MRM with clear operative bed ,no collection or masses seen on non-contrast bases.
- Right subcutaneous drain seen in-place associated with underlying mild subcutaneous surgical emphysema at right lateral chest wall with no underlying collection.
- Few scattered fine atelactatic bands likely post-inflammatory sequel.
- Few scattered small pulmonary nodules seen at whole right lung lobes as well as left lower lobe (2-4 m).
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.



Patient Name: Shadia Hosny Abd Al Azeem	Date : 21/10/2024
Exam : C.T ABDOMEN & PELVIS	Age : 65 Year(s)
Medical ID: 3890/2023	Pat. ID: 53786

Opinion:

- Few scattered small and tiny pulmonary nodules as described, likely sequel of old inflammatory process, for follow up.
- No evidence of hepatic or osseous metastatic deposits.

Dr. Marina Emad
Dr. Walid Mohamed, M.D.

Nuclear Medicine UnitPatient Name: **SHADIA HOSNY ABDEL-AZEEM**Date of Scan: **04/11/2023****ISOTOPIC Tc-99m MDP BONE SCAN REPORT****Clinical History**

A 63-year-old female patient with right breast mass ?neoplastic, for skeletal assessment.

TECHNIQUE

Anterior and posterior whole-body scintigraphy 2 hrs. following I.V injection of 20 mCi of Tc-99m M.D.P.

FINDINGS

- The scanned skeleton revealed homogenous tracer uptake, with no scintigraphic evidence of any active or cold focal lesions that may account for serious bone pathology.
- Enhanced soft tissue tracer uptake at the right foot, of benign etiology (diabetic foot), for more radiological correlation.
- Degenerative changes at different levels of the spine.
- Osteoarthritic changes over big joints.

IMPRESSION

- No definite scintigraphic evidence of any bony lesions that would be assigned to metastatic bone disease could be elicited.
- Right foot soft tissue benign-looking tracer

Thanks for reference,

Dr. Adel Khedr, M.D. (A.Kh.) Dr. Mohamed G. Hamed, M.Sc.



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : فاطمة حسن عبدالمطلب حسن

Sex:

3- Medical number:

Female

83167896

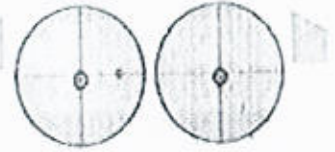
Old Med :

Old Med

389-2023

2-ID number:

Patient Age : 65 Y



Family history

Family history

Phone

Phone

01060194742

BMI

BMI

REASON OF MDT

REASON OF MDT
POST-OPERATIVE

marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

01/12/2024

Co morbidity

Co morbidity

Menopausal
status

Menopausal status
Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

Ct cap: multiple pulmonary nodule subcentemetric
for fup

BX

IDC

BX COMMENT

ER

0/8

ER COMMENT

PR

3/8

PR COMMENT

2%

Her 2 Neu

3

Her 2 Neu COMMENT

SISH

SISH

SISH COMMENT

KI67

>20

T

CT 4b

N

CN 1

M

0

Sonomammography

RT breast mass 1.5 x 1.6 mm BIRADS 5

PATIENT HISTORY

FINISHED nact tchp x 6 WITH GOOD clinical response

RT MRM : YPT0 , N0

free margin

LN 0/7

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR PORT , PHESGO X 1 YEAR

TAM X 5 YRS

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

فاطمة حسن عبدالمطلب حسن

RADIOLOGY

RADIOLOGY

[Signature]

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition